MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 24 OF 28

MFT 723

Special Populations in Therapy

ELECTIVES

1.71 units · Prerequisite: MFT 708, 714

> “"Special populations" is a compromise term.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 723 — Special Populations in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 708, 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	**Special Populations in Therapy**	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. "Special populations" is a compromise term. It names groups whom services tend to miss, misread or harm — and it trains the clinician to adapt the frame without lowering the standard of care.

How to use this book

Each week takes one population and works through four questions: what is the specific stressor, what does the evidence say helps, what must be adapted, and what should I not do? The aim is competence in adaptation, not expertise in every group.

1. Course description

The course covers families affected by forced displacement; LGBTQ+ clients and their families; people with disabilities and chronic illness; older adults; families involved with the criminal justice system; veterans and uniformed services; single-parent and grandparent-headed households; childless and infertile couples; families affected by HIV; and street-connected children.

Each carries a specific combination of stigma, legal precarity, service exclusion and clinical complexity that requires a specific adaptation of the systemic frame.

2. Learning outcomes

#	Outcome	Assessed
LO1	Identify the specific stressors and strengths of each population	Exam
LO2	Adapt assessment and intervention without diluting standards	Case study
LO3	Recognise and interrupt the ways services exclude these groups	Reflective portfolio
LO4	Work with interpreters, advocates and community organisations	Skills test
LO5	Address legal, safeguarding and ethical complexities	Viva
LO6	Evaluate the evidence for culturally adapted interventions	Essay
LO7	Design an accessible service pathway	Proposal

3. Assessment

Component	Weight	Due
Case study with adaptation plan (2,500 words)	40%	Week 9
Service pathway proposal (1,500 words)	25%	Week 12
Viva on legal and ethical complexity (20 min)	15%	Week 13
Examination	20%	Week 14

4. The fourteen-week plan

Week	Population	Adaptation focus
1	Families affected by forced displacement	Safety, loss, limbo
2	Refugees and asylum-seeking families	Legal precarity and the "second trauma"
3	LGBTQ+ clients and their families	Risk, disclosure, acceptance
4	Families with a member with a disability	Access and the social model
5	Chronic illness and palliative care families	Illness as a family member
6	Older adults and their families	Ageing, loss, elder abuse
7	Criminal justice involvement and imprisonment	Absence, stigma, re-entry
8	Families of uniformed services	Deployment, injury, silence
9	**Case study due**; single-parent and grandparent-headed households	Structure and load
10	Infertility, childlessness and reproductive loss	Stigma, grief, gender
11	Families affected by HIV	Stigma, secrecy, transmission anxiety
12	**Pathway proposal due**; street-connected children and youth	Safety, trust, systems
13	**Viva**; intersectionality and service design	Integrated planning
14	Integration and examination	Review

Week 1 — Forced displacement

The specific stressors. Loss (home, status, language, network, identity); prolonged uncertainty; trauma before, during and after flight; family separation; role reversal; and the "limbo" of waiting for legal status.

The clinical tasks. Safety and basic needs first; normalising distress without pathologising; restoring roles; supporting the family's own rituals and faith; and addressing the grief of the life left behind — often unmourned because the family is still in survival mode.

Adaptation.

- Do not require a full trauma narrative in early sessions.
- Use practical problem-solving alongside emotional work; both are therapeutic.
- Involve community and faith organisations.
- Ask about the family members who are missing — literally and emotionally.

Week 2 — Asylum-seeking families

The "second trauma" is the legal process: interviews, credibility assessments, detention, dispersal, uncertainty, and the requirement to tell the worst story repeatedly to strangers.

Clinical realities.

- Status anxiety dominates every decision: whether to report violence, whether to seek medical care, whether to disclose sexuality.
- Children may be the family's cultural and linguistic bridge.
- Court and immigration reports may be requested; the report and the therapy must be kept distinct.

What helps. Concrete assistance with documentation and access; clear, honest explanation of what you will write and to whom; liaison with legal representatives (with consent); and a therapeutic space where the person is not required to justify their deserving.

Week 3 — LGBTQ+ clients and their families

Minority stress. Prejudice, expectation of rejection, internalised stigma and concealment produce chronic stress and elevated rates of anxiety, depression, self-harm and suicide.

Family work. Families often present in crisis after disclosure, with religious and cultural conflict. The work is to: create safety for the young person; hear the parents' fear without endorsing rejection; slow the timetable (rejection, like acceptance, is a process); and set limits on harm — verbal abuse, expulsion, forced "corrective" interventions.

What not to do.

- Do not promise to change the young person, or to change the family.
- Do not require the client to come out as a condition of care.
- Do not collude with secrecy that is unsafe, or with disclosure that is premature.
- Do not refer to "the lifestyle"; use the client's language.

The clinical stance. You are not there to adjudicate theology or politics. You are there to reduce harm, improve relationships where possible, and support the person's dignity.

Week 4 — Disability

The two models. Medical (the body is the problem) and social (society's barriers are the problem). Clinically, both matter.

What families need.

- Accurate information about the condition and its trajectory.
- Help with the burden of care and the sibling experience.
- Support for the parental relationship, which is often absorbed by the caregiving.

- Advocacy with schools, employers and health services.
- Attention to the disabled person's own voice and autonomy, which families often override in the name of protection.

Adaptation. Accessible rooms; longer sessions; communication support; written materials in accessible formats; and a willingness to conduct sessions in the home when necessary.

Week 5 — Chronic illness and palliative care

The illness as a third member of the family. It takes a role, demands attention, reorganises routines and relationships.

Clinical tasks. Rebalance the couple relationship when one partner becomes a carer; address the children's understanding and fears; manage the family's beliefs about illness and fate; support decision-making about treatment; and prepare for death without abandoning hope.

Palliative and end-of-life work. Advance care planning; legacies; unfinished business; the family's conflicts about treatment; and bereavement support afterwards.

The clinician's own mortality. This work touches your own fears. Supervision is essential.

Week 6 — Older adults

Specific stressors. Retirement, bereavement, physical decline, loss of role, relocation, dependence, and the assumption by others that life is over.

Clinical presentations. Depression misdiagnosed as ageing; anxiety; cognitive change (which must be differentiated from delirium, depression, hearing loss and dementia); family conflict about care; elder abuse; and grief.

Adaptation. Slower pace; larger print; hearing support; sessions at home; involving the family in practical decisions; and taking the older person's goals as primary, not the family's convenience.

Elder abuse. Ask directly and separately: "Has anyone hurt you, taken your money, or stopped you seeing people?" Know your safeguarding route.

Week 7 — Criminal justice involvement

The absence. Imprisonment removes a person from the household while keeping them psychologically present. Children are often told a story; partners carry shame and practical burden.

Clinical tasks. Support the remaining household; address the children's understanding and stigma; prepare for release and re-entry; manage the couple's renegotiation; and address the family's history (which may include the behaviour that led to imprisonment).

Re-entry. The returning member often expects to resume an old role, while the family has reorganised without them. Naming this mismatch early prevents a second rupture.

Adaptation. Sessions in prison settings; rules about contact; careful confidentiality (the prison is not a confidential setting); and coordination with probation or social services.

Week 8 — Uniformed services

The specific culture. Stoicism, mission focus, hierarchy, and a strong norm against help-seeking.

Clinical presentations. Deployment and reintegration; moral injury; PTSD; substance use; domestic violence; and the family's secondary traumatisation.

Adaptation. Frame treatment in terms the culture respects (performance, mission readiness, the family's stability); involve the unit where appropriate; and address the couple's shift in roles during absence and return.

Week 9 — Single-parent and grandparent-headed households

The specific stressors. Role overload; financial strain; the absent parent's shadow; stigma; and children's questions.

Strengths. Many single-parent households function very well; treat the structure as a structure, not a deficit.

Grandparent-headed households. Grief (for the parent, often due to death or incapacity); the generation gap; legal status (custody, maintenance); health limitations; and the children's loyalty conflicts.

Adaptation. Practical support first; a clear parenting plan; support for the child's contact with the absent parent where safe; and attention to the carer's own health.

Week 10 — Infertility and childlessness

The cultural weight. In many settings, childlessness carries severe social stigma, blamed disproportionately on women, with consequences for marriage stability and family standing.

Clinical themes. Grief (often disenfranchised); shame; blame and the couple's conflict; pressure from extended family; religious meaning; treatment decision-making and its cost; and the couple's sexual relationship, which becomes medicalised.

The clinical stance. Do not assume that parenthood is the goal for every couple. Ask what a good life looks like for them. Name the social pressure as external, not personal failure.

Week 11 — Families affected by HIV

The clinical realities. Stigma and secrecy; disclosure decisions (to partner, children, family, church); medication adherence; transmission anxiety; bereavement and orphanhood; and the intersection with poverty and gender inequality.

Disclosure work. Help the person decide who, when and how; rehearse; prepare for different responses; and ensure the children's questions are answered at their developmental level.

Adaptation. Confidentiality is paramount in small communities; coordinate with HIV services carefully; address the family's beliefs about illness and blame; and support the "worried well" partner as well.

Week 12 — Street-connected children and youth

The specific realities. Survival needs first (food, safety, shelter); mistrust of adults and institutions; peer networks that function as family; substance use; exposure to violence and exploitation; and legal vulnerability.

What helps. Outreach and continuity; a trusted adult who does not disappear; harm reduction; family reunification work only where it is safe; and collaboration with community organisations, not only clinics.

What harms. Coercive removal; moralising; short-term projects that end abruptly; and treating the peer group as pathology when it is often the only protection available.

Week 13 — Intersectionality and service design

The design questions.

1. Who does not come to our service, and why?
2. What are the access barriers — cost, transport, language, hours, stigma, documentation, disability?
3. Who is the first point of contact, and are they trained?
4. What does the physical environment communicate?
5. What happens when a client needs an interpreter, a wheelchair, a child-friendly space, or a private entrance?
6. How do we measure whether these groups are actually being served?

A pathway that works typically includes: community outreach; a named link worker; flexible appointments; interpreters; transport support; plain-language materials; and a feedback loop with the communities served.

> **PROPOSAL (25%)**
> A service pathway for one under-served population, with access analysis, staffing, and outcome measures.

Week 14 — Integration

The generalizable skills. Ask rather than assume; adapt the frame, not the standard; work with community organisations; attend to legal precarity; protect confidentiality in small communities; and examine your own discomfort.

The generalizable caution. "Special population" can become a way of othering. Every family is specific; the skill is in the specificity, not in the label.

Glossary

Asylum seeker — a person seeking protection who has not yet been granted refugee status. **Disenfranchised grief** — grief that is not socially recognised or supported. **Minority stress** — chronic stress from stigma, prejudice and concealment. **Moral injury** — psychological distress from acting against one's values in service. **Social model of disability** — disability located in societal barriers rather than the body.

Self-assessment

Multiple choice

1. The "second trauma" for asylum seekers refers to: (a) the original event (b) the legal process (c) the journey (d) therapy

2. Family work after a young person's coming out should first: (a) change the young person (b) create safety and hear the parents' fear (c) enforce acceptance (d) require celibacy

3. The social model of disability locates disability in: (a) the body (b) societal barriers (c) genetics (d) the family

4. Elder abuse should be: (a) assumed unlikely (b) asked about directly and separately (c) ignored (d) reported without consent always

5. On release from prison, families commonly: (a) resume old roles easily (b) have reorganised without the returning member (c) refuse contact (d) need no support

6. Childlessness in many settings is: (a) neutral (b) severely stigmatised and blamed on women (c) rare (d) a legal matter

7. For street-connected youth, the first priority is: (a) family therapy (b) survival needs and a trusted adult (c) diagnosis (d) schooling only

8. Minority stress explains elevated rates of: (a) income (b) anxiety, depression and self-harm (c) physical fitness (d) employment

9. Effective pathways for under-served groups begin with: (a) more clinics (b) access analysis and community outreach (c) longer waiting lists (d) more diagnosis

10. The risk of the term "special populations" is: (a) inaccuracy (b) othering and stereotyping (c) legality (d) cost

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name four specific stressors of forced displacement and one clinical task for each.
2. What are three "do nots" in family work after a coming-out disclosure?
3. How does the social model of disability change your assessment?
4. What must a service pathway include for under-served populations?
5. Why is confidentiality harder in small communities, and what do you do about it?

Essay (choose one, 2,000 words)

1. "Adaptation without dilution." Discuss with reference to one special population.
2. Critically evaluate the evidence for culturally adapted family interventions.
3. Design an accessible family therapy service for one excluded group in your context.

Further reading

- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Papadopoulos, R. K. (Ed.). (2002). Therapeutic Care for Refugees. Karnac.
- Laing, L., & Topping, K. (2019). Family Therapy with Older Adults. Routledge.
- Lev, A. I., & Sennott, S. L. (2019). Gender and Sexuality in Family Therapy. Routledge.
- Miller, G. (2011). Management of Alcohol and Drug Problems. (For harm-reduction principles in outreach work.)

Next: MFT 724 — Family Law and Advocacy